

Research on the Demand for Building Cognitive Health-Friendly Environments

—A Case Study of Shanghai

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Abstract: Against the backdrop of deepening population aging, the number of people living with mild cognitive impairment and dementia continues to grow. With over 90 percent of older adults with cognitive impairment aging in place in their original communities, the living environment has become a critical factor in delaying cognitive decline and alleviating the burden of home-based care. Drawing on a large-scale questionnaire survey, this study precisely delineates the core demands of elderly residents in Shanghai regarding the development of cognitive health-friendly environments. The findings reveal that: in terms of the physical environment, older residents prioritize basic safety and accessibility far above the construction of specialized functional zones; at the service delivery level, community respite care is scarce and support for family caregivers remains inadequate; regarding social support networks, there is an urgent need to expand professional volunteer and service teams; and in terms of payment for services, elderly residents rely heavily on government subsidies and show a strong preference for public-interest and mutual-aid models. Accordingly, this report puts forward recommendations across three dimensions—the physical environment (emphasizing low-cost, incremental renewal), the social environment (building networks of trust), and the service system (establishing an integrated screening-intervention-care continuum)—and formulates a basic needs checklist for building cognitive health-friendly environments to inform decision-making by community management authorities.

Keywords: cognitive health; older residential communities; physical environment; service delivery; social support

I. Research Background

Shanghai currently has 5.6805 million registered residents aged 60 and above. The diagnosed prevalence of cognitive impairment among community-dwelling older adults ranges from 4.63 percent to 6.5 percent, affecting approximately 300,000 individuals. The high-risk population with mild cognitive impairment accounts for 14 percent to 15.7 percent, bringing the total number of older adults with cognitive function impairment to approximately 1.15–1.20 million. Prevalence rises markedly with age: over 20 percent of those aged 85 and above exhibit cognitive symptoms. Yet diagnosis rates remain alarmingly low—only 14 percent for mild cases and 34 percent for severe cases—leaving a substantial number of patients unidentified. The number of at-risk older adults officially registered and documented falls far short of the actual prevalence, indicating a large proportion of hidden, undetected cases.

As population aging intensifies, the population living with mild cognitive impairment and dementia continues to expand. Over 90 percent of older adults with cognitive impairment age in place in their original communities, making the community environment and integrated care system pivotal in delaying cognitive decline and mitigating the challenges of home-based care. However, older residential communities constitute the primary living areas for Shanghai's elderly population: in public housing estates within the Inner Ring Road, residents aged 60 and above typically account for 45–65 percent of the permanent population, with a high concentration of adults aged 80 and above, those living alone, and those with cognitive impairment—where the mismatch between age-friendly supply and demand is most acute. Approximately 14,000 older residential communities in Shanghai have been included in the renovation inventory, accounting for nearly 40 percent of all residential communities citywide. These communities generally exhibit age-friendly deficiencies in three major areas: First, lack of barrier-free access—over 90 percent of multi-story public housing and old alleyway houses lack elevators, making it extremely difficult for frail elderly residents to go outdoors;

stairways consist entirely of steps without ramps, posing a very high risk of falls. Second, deficiencies in age-friendly public spaces—uneven road surfaces, mixed pedestrian-vehicle traffic, absence of continuous barrier-free walkways, insufficient resting seats; hardened green spaces lacking shaded rest areas and age-appropriate fitness equipment; dim lighting and incomplete surveillance coverage, heightening the risk of wandering among people with dementia. Third, shortcomings in supporting facilities and services—older communities suffer from a significant shortage of embedded elderly care service stations.

Against this backdrop, this study is grounded in the realities of older adults' living environments and cognitive health in Shanghai. It seeks to gain an in-depth understanding of the genuine needs of diverse elderly groups regarding early identification, daily care, and social inclusion, with the aim of precisely identifying the most pressing demands of elderly residents for cognitive health services and thereby facilitating the formulation of practical, highly standardized guidelines for building cognitive health-friendly environments.

II. Research Process and Analysis

To accurately assess the cognitive health awareness and practical needs of elderly residents in Shanghai, this study designed and administered a questionnaire survey targeting community-dwelling active older adults, aiming to present the core demands for building cognitive health-friendly environments through quantitative data.

1. Questionnaire Distribution and Sample Collection

The questionnaire survey was conducted from June to July 2026, distributed and collected electronically via the Wen Juan Xing (Questionnaire Star) platform. For distribution, electronic questionnaires were primarily entrusted to professional community social workers and volunteers, who organized eligible active older adults in their vicinity to complete the survey.

To balance representativeness and practical feasibility, three eligibility criteria were applied to respondents: first, respondents had to be residents aged 60 or above with normal mobility

and comprehension abilities to ensure they could complete the questionnaire independently and accurately; second, a target ratio of at least one male per four respondents was set to balance the gender structure and prevent overrepresentation of females; third, respondents were selected to reflect the general elderly population rather than concentrating on specific groups such as community activists. Furthermore, considering the advanced age of the target population, the questionnaire was fully adapted for elderly respondents, featuring large-font formatting and a length designed to be completed within approximately ten minutes to minimize response burden.

Under this distribution mechanism, 769 valid questionnaires were ultimately collected. With a broad geographic coverage and a relatively balanced demographic structure, the sample adequately reflects the general conditions of community-dwelling active older adults regarding cognitive health literacy, spatial environment demands, and stigma, providing reliable primary data for subsequent needs analysis.

2. Basic Demographic Characteristics of the Sample

In terms of age distribution, the sample had a mean age of 70.5 years and a median of 70 years, ranging from 60 to 97 years, with the majority falling in the young-old and middle-old categories. Specifically, those aged 60–69 accounted for 44.6 percent and those aged 70–79 for 48.8 percent, together comprising over 90 percent (93.4 percent) of the sample; adults aged 80–89 constituted 5.6 percent, and those aged 90 and above 1.0 percent. This distribution is consistent with the sampling frame targeting "active older adults"—respondents with normal mobility and comprehension—which naturally underrepresents the oldest-old and those with functional limitations. The sample is thus more representative of older adults who retain independent living capabilities and fall within the key target group for cognitive health interventions.

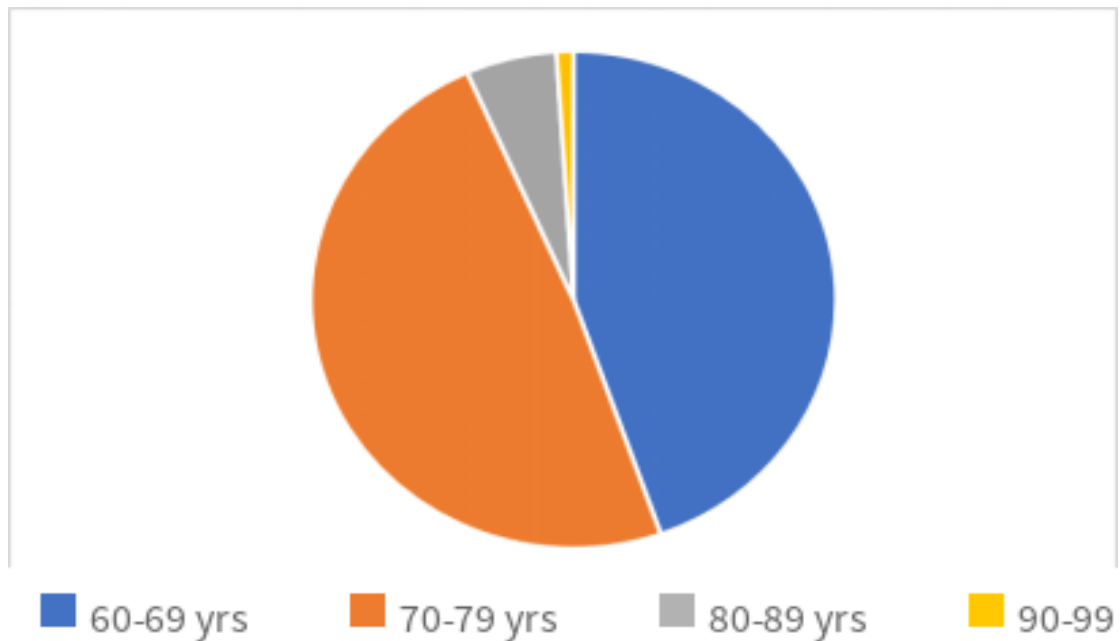


Figure 1. Age Distribution of the Sample

Regarding gender composition, females constituted 60.1 percent of the sample and males 39.9 percent, with a slight overrepresentation of women. This pattern is attributable to the gender structure of the elderly population and the relatively higher propensity of women to participate in community activities. Nevertheless, the approximate 2:3 male-to-female ratio was consistent with research expectations.

Regarding physical functioning, 82.3 percent of respondents were fully independent without mobility limitations, 15.7 percent were independent but experienced difficulties with physical activities, and only 2.0 percent required assistive devices. The overall high level of self-care capability again confirms the active nature of the sample. It is noteworthy, however, that nearly one-fifth of older adults already experience mobility difficulties or require assistance; this group has particularly urgent needs for age-friendly environmental features such as pedestrian pathways, resting seats, and barrier-free access within residential communities.

In terms of living arrangements, the proportion living with a spouse was highest at 72.7 percent; 30.4 percent lived with children, 8.3 percent with grandchildren, and 11.7 percent

lived alone. Fewer respondents were cared for by domestic workers or caregivers (0.9 percent) or lived with other relatives (0.9 percent). This structure indicates that most older adults live with family companionship, with spouses and children serving as the primary source of family support for cognitive health—underscoring that caregiver support and family empowerment should be key components of the service system. Meanwhile, the over 10 percent who live alone represent a priority group for early cognitive risk identification and regular monitoring, warranting more targeted community attention.

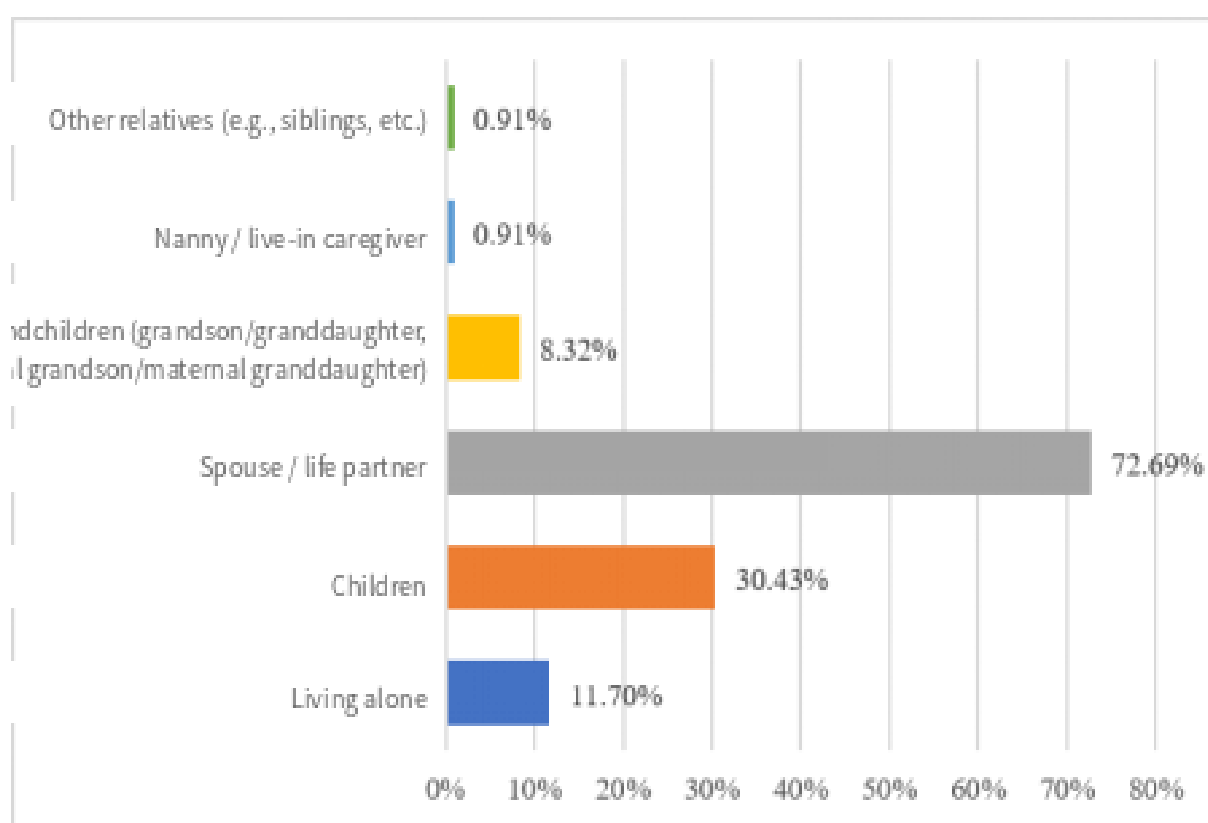


Figure 2. Living Arrangements of Respondents

Regarding monthly disposable income, the sample overall exhibited a low-to-middle income level with a relatively dispersed distribution: those with incomes below 5,000 yuan collectively accounted for 74.4 percent, while those with incomes above 10,000 yuan constituted only 9.0 percent. This income structure suggests that the spending capacity of elderly residents in the surveyed areas is limited. The pricing and payment models for

cognitive health-related services must therefore take their affordability into full account; publicly funded, universally accessible services supported by government subsidies are preferable, while market-based, high-fee service models currently have limited acceptance among residents.

Overall, the sample was predominantly composed of active, independently living older adults aged 60–79, with a higher proportion of females, family support primarily from spouses and children, and a generally low-to-middle income level. This group represents the core target for "upstream prevention" in cognitive health; their living environments and economic characteristics are typical of older residential communities, which establishes realistic parameters for the subsequent needs analysis.

3. Self-Perceived Cognitive Health and Screening Participation

More than half of elderly residents held optimistic perceptions of their own cognitive health. Data showed that when reflecting on changes in their cognitive abilities over the preceding two years, 57.09 percent reported experiencing "none of the above" changes, indicating generally positive self-assessments. However, among those who noticed changes, "finding it much harder than before to learn to use new tools or devices" ranked highest (20.16 percent), followed by "reduced interest in things previously enjoyed" (17.95 percent) and "problems with everyday memory and thinking" (12.09 percent). These findings point to a dual challenge facing active older adults: on one hand, in an increasingly digital era, mild declines in executive function and learning ability are often first exposed through the "digital divide"; on the other hand, reduced emotional motivation—such as diminished interest—a common early concomitant symptom of cognitive decline, is already being acutely perceived by residents in their daily lives.

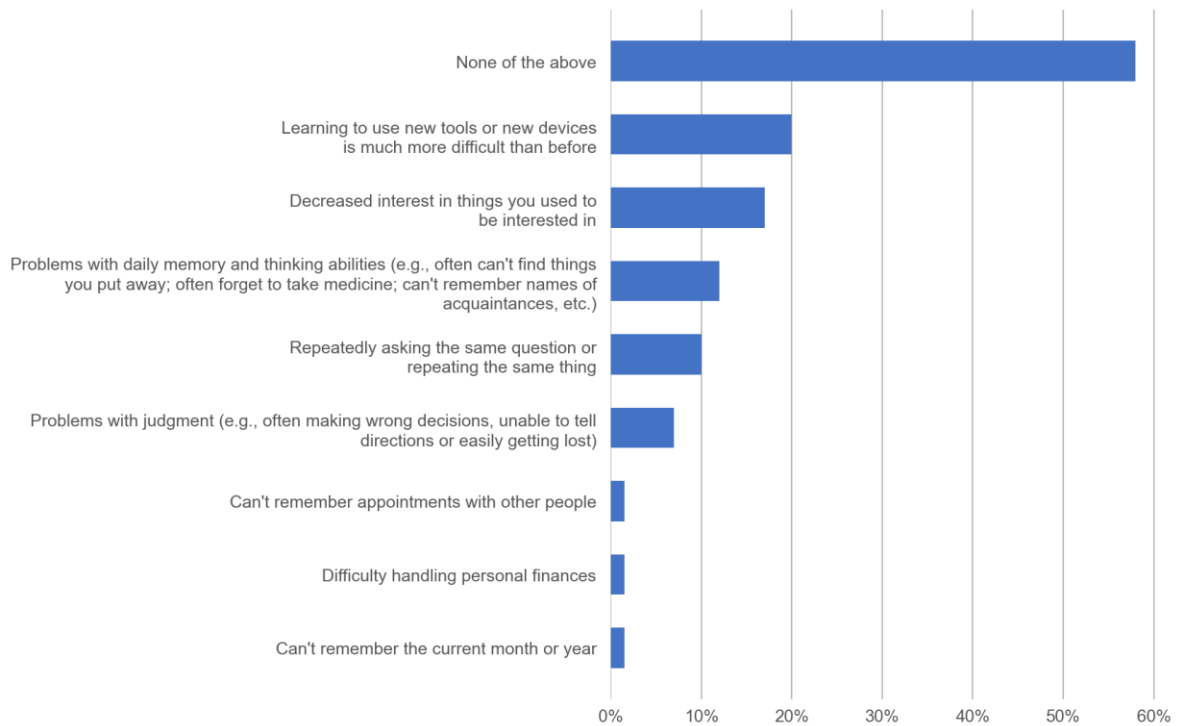


Figure 3. Distribution of Respondents' Self-Perceived Cognitive Health Status

The overall participation rate in cognitive screening among elderly residents was relatively low, yet their awareness and initiative in daily self-directed prevention were comparatively strong. The survey revealed that only 37.32 percent of respondents had ever participated in community cognitive screening. More critically, among those who had been screened, as many as 94.43 percent received a "low risk (cognitively normal)" result, while only 5.23 percent and 0.35 percent were identified as medium-risk and high-risk, respectively. This indirectly confirms that those who proactively participate in screening tend to be the most active and healthy "community enthusiasts," while the truly high-risk groups often remain hidden beyond the reach of screening networks. In contrast to the low medical screening participation rate, only 5.20 percent of older adults reported taking no special preventive measures in daily life. The overwhelming majority preferred to promote cognitive health through lifestyle-oriented, de-medicalized approaches, such as maintaining regular routines

and balanced diets (60.21 percent), engaging in physical exercise (57.35 percent), and actively sustaining social connections (52.93 percent). This suggests that future intervention strategies should not be confined to a medical perspective; rather, they should align with older adults' daily habits, integrating non-pharmacological interventions naturally into the community's everyday leisure and social activity networks.

4. Dementia Literacy and Stigma

Elderly residents' knowledge of dementia exhibited a structural divide: "strong everyday knowledge, but deficient understanding of pathology and intervention." In the dementia literacy test, residents demonstrated relatively good grasp of observable, life-related knowledge—for instance, correctly identifying that "exercise benefits people with dementia" (72.0 percent accuracy) and that "symptoms of depression can be mistaken for dementia" (52.8 percent accuracy). However, when it came to in-depth pathological classifications or professional treatment approaches, knowledge gaps widened considerably. For example, only 3.8 percent correctly identified the false statement that "vascular dementia is the most common type" (meaning the vast majority either believed it to be true or did not know); similarly, only 5.5 percent could accurately refute the misconception that "medication is the most effective treatment for behavioral symptoms." Such misconceptions may lead older adults to seek medical care unnecessarily or to over-rely on pharmacological interventions when faced with actual symptoms.

Table 1. Accuracy Rates on Dementia Literacy Questions

Question Item	Accuracy Rate	Question Item	Accuracy Rate
Dementia is a normal part of the aging process	5.6%	People with advanced dementia frequently communicate through body language	8.2%
The most common type of dementia is reversible	13.9%	When a person with dementia exhibits atypical behavior, it usually indicates an unmet need	48.5%
Vascular dementia is the most common type of dementia	3.8%	Medication is the most effective treatment for behavioral symptoms of dementia	43.0%
Most types of dementia do not typically shorten lifespan	9.4%	People with dementia usually have no difficulty making decisions	5.5%
High blood pressure increases an individual's risk of developing dementia	52.8%	Medication is the most effective way to treat behavioral symptoms in people with dementia	36.7%
Symptoms of depression can be mistaken for symptoms of dementia	50.2%	People with advanced dementia may have difficulty speaking	62.7%
Exercise is beneficial for people with dementia	72.0%	People with dementia typically have difficulty learning new skills	68.1%
When a person with dementia becomes confused, it is important to correct them	18.1%	Eating difficulties typically occur in the later stages of dementia	55.5%

The community atmosphere was generally accepting, with overt stigma at a low-to-moderate level, though public education still needed improvement in practicality and precision. The mean self-rated stigma score among respondents was 3.96 (on a 0–10 scale, median 3), and 46.42 percent of older adults explicitly stated that there was "no prejudice and everyone is very accepting" toward people with dementia in their community. Nevertheless, it cannot be overlooked that nearly 30 percent of residents perceived occasional or widespread discrimination, indicating that implicit psychological defenses persist. When evaluating shortcomings of current community education, the top-ranked issue was "lack of early

prevention guidance for at-risk populations" (59.29 percent combined), followed by "too few educational activities and limited information channels" (41.22 percent) and "educational content too professionally jargon-heavy to understand" (40.70 percent). This clearly demonstrates that the current education model—predominantly static display boards and one-way dissemination—can no longer meet residents' needs. What older adults require is not obscure medical terminology, but highly practical, actionable guidance addressing questions such as "How can I prevent this?" and "Where do I seek help if I notice problems?"

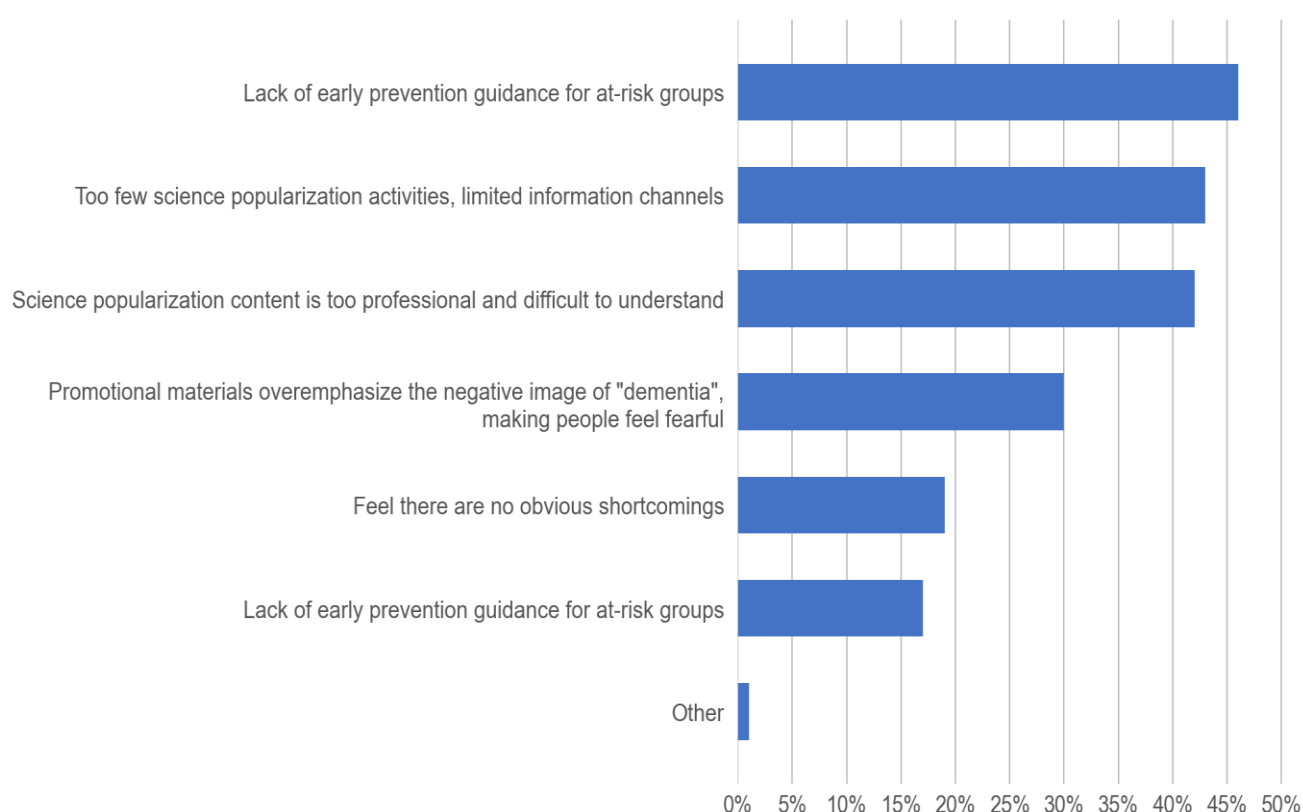


Figure 4. Distribution of Respondents' Views on Shortcomings in Current Community Education

5. Community Spatial Environment Experience and Renovation Demands

The interpersonal atmosphere in community public spaces was highly rated, yet there were notable deficiencies in creating age-friendly "sense of place" and emotionally supportive elements. In spatial experience evaluations (on a 1–5 scale), soft social environment items

scored impressively: "community staff or neighbors are willing to offer help" (4.36) and "frequently encountering acquaintances who stop to chat" (4.30) received the highest scores. However, items related to refined physical environments ranked lowest: "there is a special or familiar place in the community I like to go to" (3.89) and "corner arrangements evoke pleasant memories" (3.90) both fell below the 4-point threshold. This indicates that current physical environments in residential communities largely remain at the level of basic landscaping and cleanliness, lacking in-depth design for spatial orientation, emotional comfort, and sense of belonging for older adults with cognitive impairment.

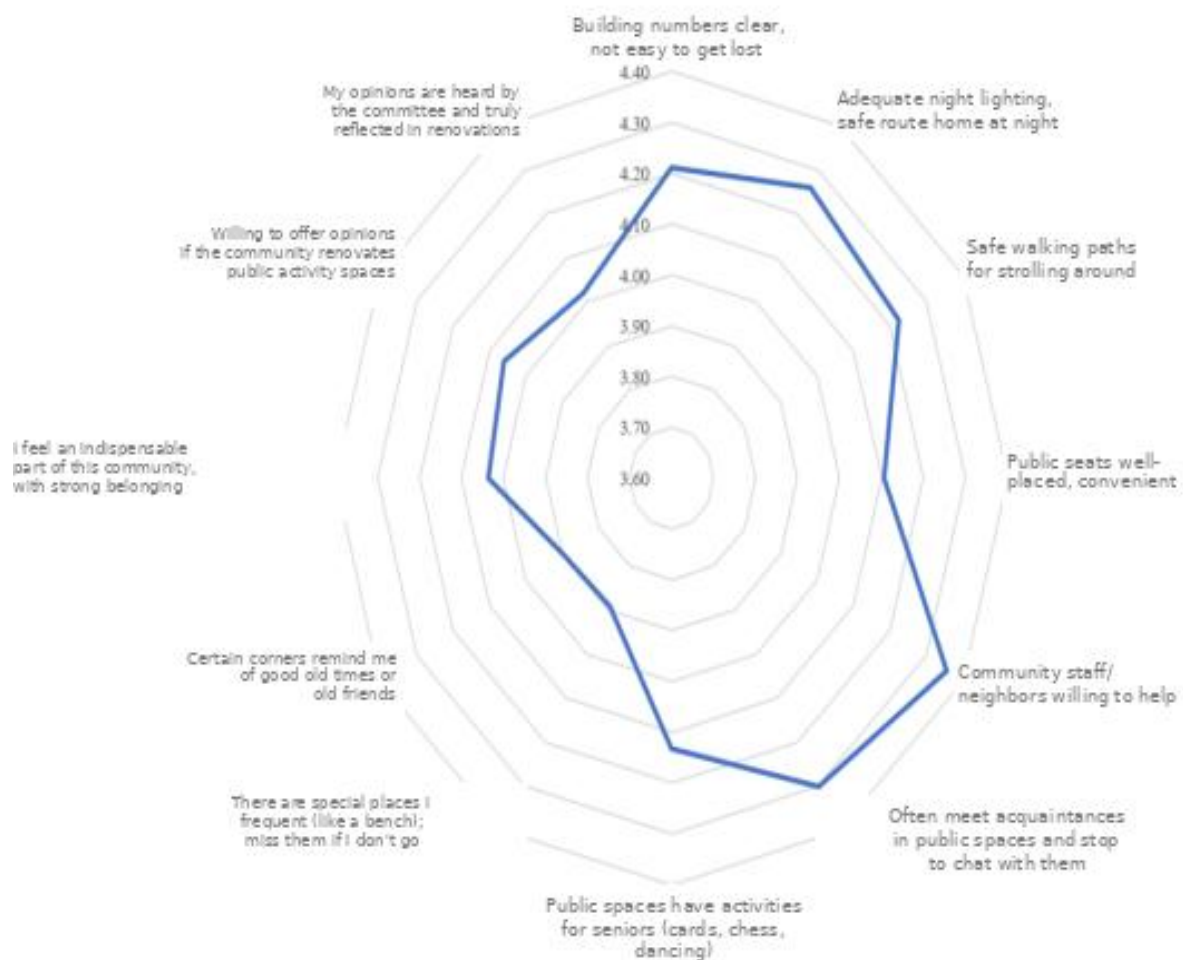


Figure 5. Radar Chart of Spatial Environment Experience in Respondents' Communities

High-frequency daily circulation routes represent the top priority for physical environment renovation, with demands for basic safety and accessibility far exceeding demands for specialized functional zones. In terms of spatial use frequency, corridors and pedestrian walkways were the spaces most frequently used by older adults on a daily basis, with "almost daily use" rates as high as 69.18 percent and 56.70 percent, respectively. When asked about "activity areas they wished existed but were currently lacking," "designated walking or strolling routes" (44.99 percent) and "sheltered leisure areas for neighbors to chat" (39.14 percent) ranked highest.

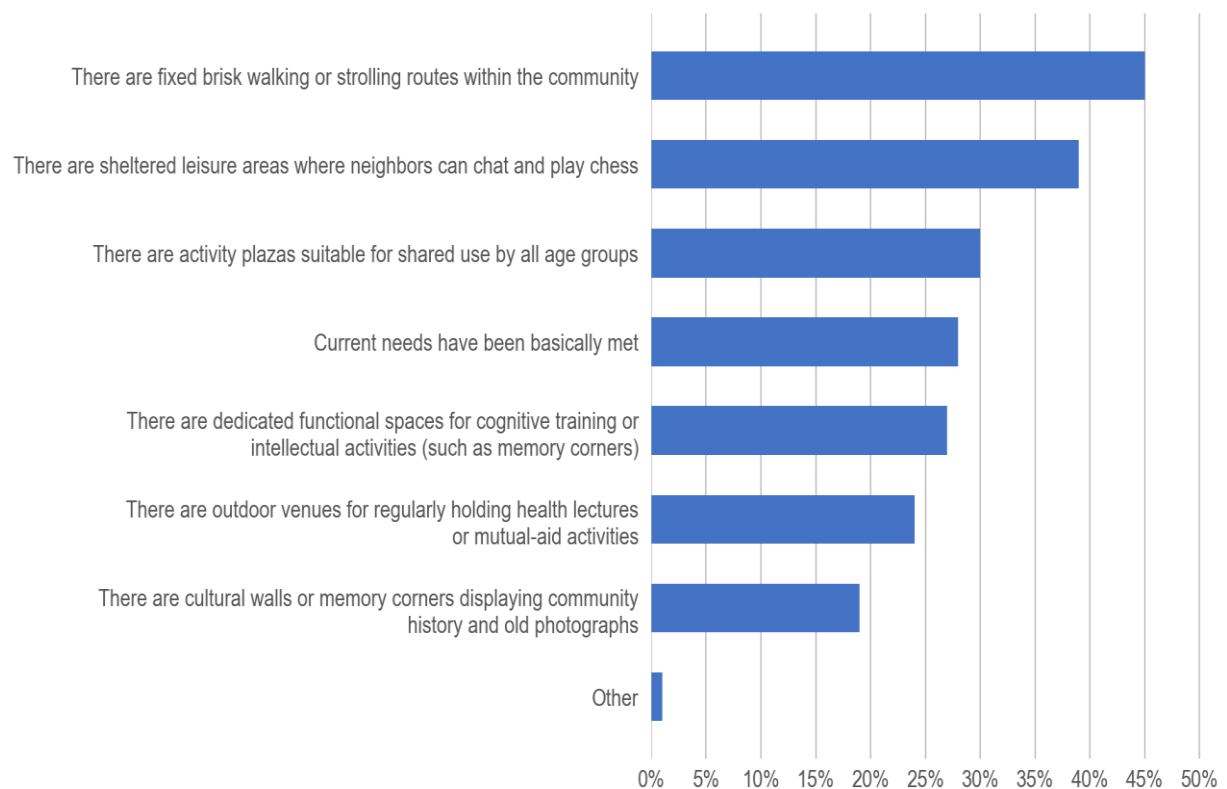


Figure 6. Distribution of Demands for Public Space Development in Respondents' Communities

Furthermore, when selecting the "top two priorities for improvement," "safer, smoother walking paths" (47.20 percent), "more resting seats" (29.78 percent), and "clearer pictorial wayfinding signage" (28.22 percent) emerged as the three most consensual priorities. By

contrast, specialized cognitive training zones or reminiscence corners ranked relatively low (both below 15 percent). These data clearly indicate that for older adults who venture outdoors, smooth and safe pathways, readily available seating for rest, and signage that prevents getting lost constitute the "lifeline" that enables them to leave their homes and maintain social engagement. The development of cognitive health-friendly environments must first and foremost address the universal age-friendly needs for safety and barrier-free access among the elderly population.

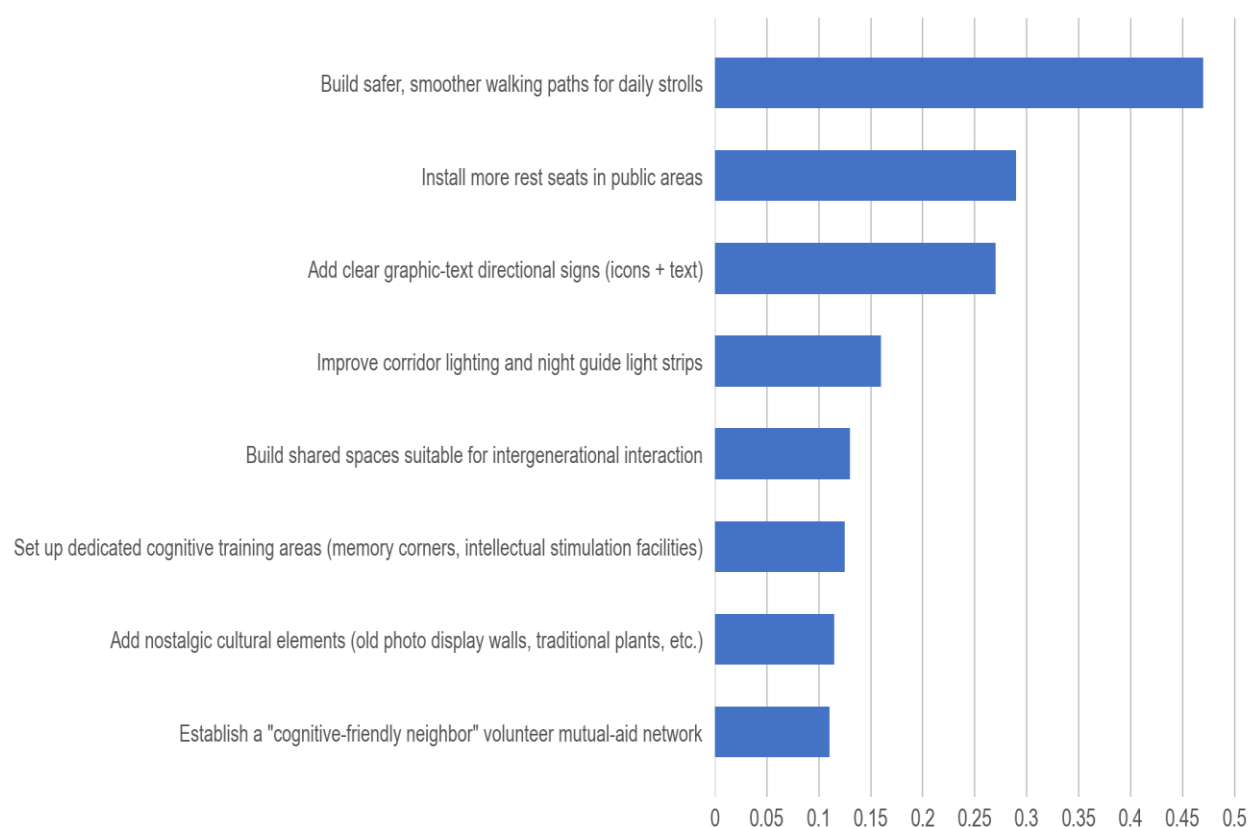


Figure 7. Most Urgent Aspects of Cognitive Health-Friendly Environment Renovation as Perceived by Respondents

6. Cognitive Health Service Demands and Willingness to Pay

Elderly residents generally held high expectations for various cognitive health support services and showed a marked preference for public-interest and mutual-aid models in service provision. For all seven categories of services listed in the questionnaire—including cognitive

screening, training groups, and health lectures—the combined proportion of respondents who considered them "very helpful" or "somewhat helpful" exceeded 80 percent, indicating broad latent demand for services. When it came to introducing social resources from different backgrounds, traditional medical research institutions or commercial organizations were not the first choice. Respondents were most willing to participate in activities organized by "voluntary cognitive health service teams for public welfare" (63.07 percent) and "mutual-aid organizations for older adults" (45.77 percent). This reflects that within the close-knit social networks of communities, active older adults prefer to receive companionship and interventions in a relaxed, mutual-aid atmosphere rather than being treated as "patients"; informal support networks based on neighborly trust are most acceptable to them.

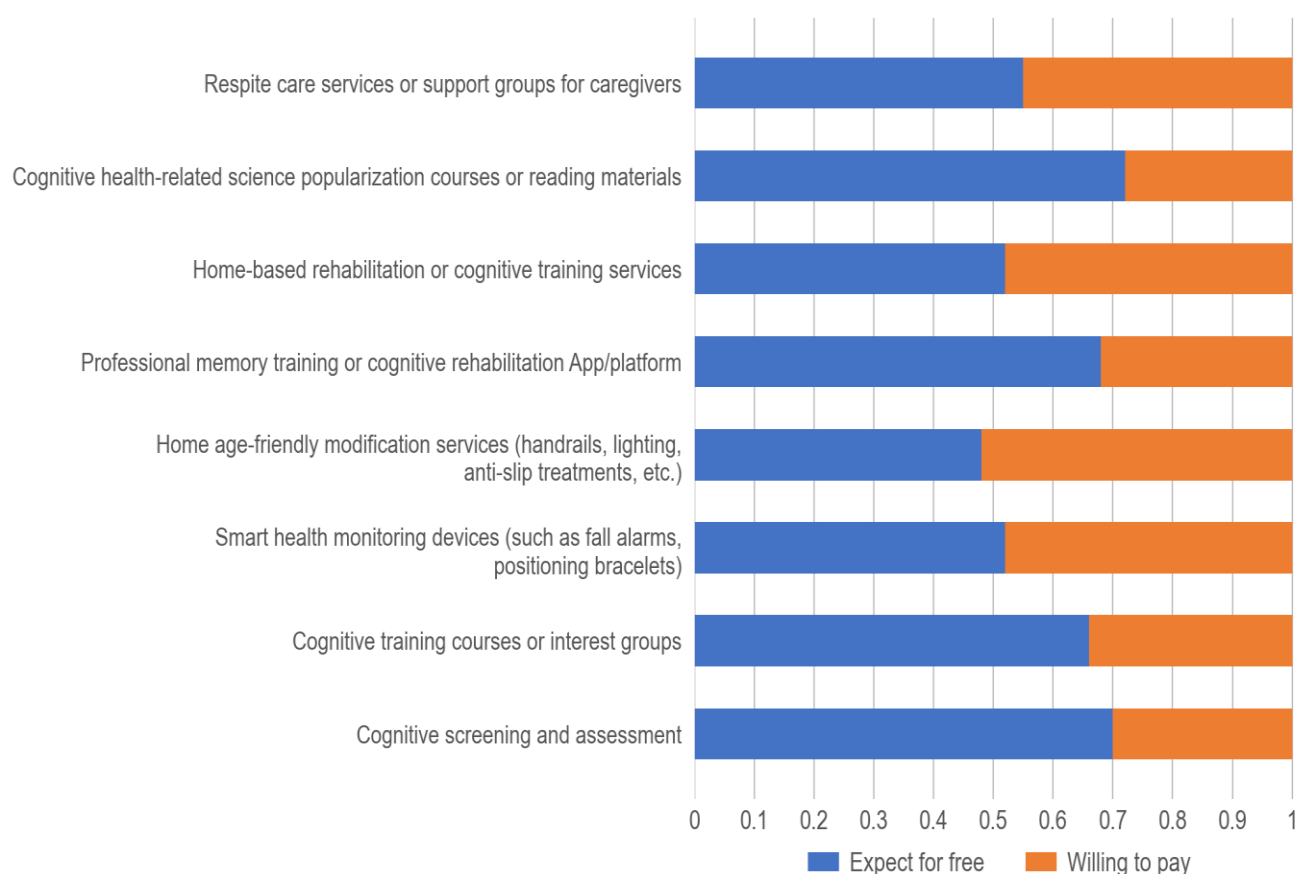


Figure 8. Respondents' Willingness to Pay for Cognitive Health-Related Services or Products

Older adults demonstrated a relatively strong willingness to invest in their own health, yet their actual payment behavior was characterized by heavy reliance on government support. The survey showed that in terms of consumption philosophy, 53.97 percent agreed with the statement "prioritize ensuring my own health and quality of life; consider children only if there is surplus capacity," reflecting a proactive attitude toward self-health management. However, regarding willingness to pay for specific services, the proportion expecting services to be free was very high: cognitive health educational materials (71.52 percent) and cognitive screening (68.92 percent) were regarded as services that should most appropriately be borne by public finance. By comparison, services involving hardware investment or tangible outputs—such as "home-based age-friendly renovation" (48.37 percent expecting free provision) and "home-based rehabilitation training" (51.11 percent)—showed slightly higher willingness to pay. In terms of overall payment model preferences, as many as 90.38 percent of respondents favored "full government subsidy with free personal use" or "government subsidizes the majority, with individuals paying a small portion." This apparent contradiction suggests that, given the reality of numerous older residential communities and the limited spending capacity of elderly residents in Shanghai, the development of cognitive health-friendly environments must retain its public-interest orientation.

III. Research Conclusions

Drawing on the quantitative analysis of the questionnaire survey, in-depth interviews with 14 professional social workers, and lessons from domestic and international practices in building dementia-friendly communities, this study proposes recommendations for the core components of developing cognitive health-friendly communities across three dimensions: the physical environment, the social environment, and the service system.

1. Physical Environment: Emphasizing "Low Cost" and "Incremental Renewal"

Guidelines for the physical environment follow an "outside-to-inside" circulation sequence

(main entrances and exits—pathways and buildings—outdoor activity spaces—indoor spaces), prioritizing the elimination of safety hazards before enhancing cognitive-friendly qualities.

First, main entrances and exits. Emphasis is placed on barrier-free access and clear directional guidance. High-contrast, large-font site plans marking key locations are required. For older residential communities, temporary ramp plates and similar solutions are encouraged as alternatives to costly permanent barrier-free ramps.

Second, pathways and buildings. Key attention is given to age-friendly lighting and signage systems. Main pathways must maintain uniform illumination and avoid glare; multi-directional pictorial signage should be installed at building entrances and turning points, continuous non-slip handrails and step edge markings should be installed in stairwells, and color contrast should be used to aid orientation.

Third, outdoor activity spaces. The development of cognitive-friendly outdoor circulation routes is advocated, using colored ground lines and differentiated paving materials to delineate walking paths and form continuous loop routes. Existing green spaces and fitness areas should be fully utilized, with additional seating installed at appropriate heights.

Fourth, indoor spaces. Rather than mandating the construction of dedicated new facilities, communities are encouraged to repurpose idle public housing, property management spaces, or existing activity rooms, creating multi-use cognitive-friendly activity hubs through wall-mounted features (memory walls, game boards) and movable furniture; essential safety monitoring devices should also be installed in the homes of high-risk older adults.

2. Social Environment: Building a Trust Network of "Understanding, Acceptance, and Support"

First, establishing volunteer teams. Enthusiastic residents and younger elderly individuals within the community should be identified to serve as cognitive health volunteers; through training, professional support can be deeply integrated with neighborhood mutual aid.

Second, promoting innovative community integration. Collaborating with nearby

convenience stores, markets, and pharmacies to provide staff training, set up priority seating, and simplify payment processes. Intergenerational programs should also be encouraged, organizing school students to engage in cross-generational interactions to thoroughly break the social isolation experienced by people with cognitive impairment.

3. Service System: Establishing an Integrated "Upstream Screening–Midstream Intervention–Downstream Care" Continuum

First, establishing community cognitive-friendly service points. Based in existing public facilities, service points should be designated with distinct zones for screening, intervention, and consultation.

Second, public education and awareness. Permanent information boards should be installed at main community entrances, elevator lobbies, and other locations, disseminating prevention knowledge through illustrated materials; stigma reduction should be promoted through themed events.

Third, risk screening and precision management. The accessibility of screening should be improved, and tiered precision management implemented based on screening results: Category A (normal) individuals are included in health promotion programs; Category B (medium-to-high risk) individuals receive brain health records and interventions; Category C (suspected condition) individuals are provided with referral pathways.

Fourth, diversified non-pharmacological interventions. Regular activities such as memory training and handicraft-making should be offered at service points; for high-risk or diagnosed older adults, individualized case management and group interventions such as music therapy and reminiscence therapy should be provided, with regular outcome evaluations.

Fifth, family empowerment and caregiver support. This is the cornerstone of the service system. Tiered skills training covering basic care, advanced nursing, and psychological adjustment should be provided to family caregivers; mutual support groups and respite care networks should be established; caregivers should be included in health check-ups and

psychological interventions to comprehensively alleviate the family caregiving burden.

Sixth, professional care and referrals. The supply of various care services—including day care and home-based services—should be expanded, and partnerships with specialized hospitals should be established to implement family doctor contracting and extended medical services.

Table 2. Basic Needs Checklist for Building Cognitive Health-Friendly Environments

I. Physical Environment Renovation

No.	Task Module	Specific Operational Requirements
1	Entrances, Exits and Access Control	1. Barrier-free access: Entrances with elevation differences must be equipped with ramps or temporary ramp plates. 2. Age-friendly access control: Controls must be positioned at a height convenient for standing elderly users and wheelchair operation. 3. Smart monitoring: Main entrance surveillance should automatically identify individuals with cognitive impairment to prevent wandering.
2	Age-Friendly Signage and Wayfinding	1. Site plans: Large-font, high-contrast community site plans and maps of cognitive-friendly facilities should be posted prominently at main entrances. 2. Enlarged building numbers: All building numbers should uniformly adopt "bold large font + high-contrast colors." 3. Intersection signage: Multi-directional directional signs should be installed at key intersections.
3	Pathways and Stairways	1. Slip and fall prevention: Non-slip edge markings should be installed on outdoor stair treads; continuous non-slip handrails should be installed on stairways. 2. Age-friendly lighting: Basic safety lighting must be maintained during late-night hours.
4	Public Activity Spaces	1. Outdoor circulation routes: Fitness walking paths should be marked with colored ground lines or differentiated paving. 2. Outdoor facilities: Seats at heights suitable for standing up should be added, and surfaces should be treated for slip resistance. 3. Indoor micro-renovation: Existing elderly activity rooms and similar spaces should be adapted to create intervention zones.

II. Social Environment Support

No.	Task Module	Specific Operational Requirements
5	Volunteer Teams	1. Recruitment and team building: Active "cognitive health volunteers" should be recruited from enthusiastic residents and healthy younger elderly community members. 2. Professional training: Volunteers must receive no less than eight hours of systematic professional training.
6	Friendly Business Alliance	1. Merchant alliance: Partnership agreements should be signed with nearby markets, convenience stores, and pharmacies, with unified friendly signage displayed.

		2. Age-friendly services: Merchants commit to providing priority seating, large-font price tags, and staff trained to contact the community when encountering elderly individuals who appear lost.
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III. Cognitive Health Service System

No.	Task Module	Specific Operational Requirements
7	Service Points and Public Education	1. Service point configuration: Service points should be established with designated areas for initial screening, intervention, and private consultation. 2. Resource publicity: Hotline numbers and referral resource maps should be publicly displayed. 3. Regular education: Fixed information boards should be placed in high-traffic areas.
8	Regular Risk Screening	1. Proactive initial screening: Annual initial screening should be offered to willing residents aged 60 and above. 2. Categorized record-keeping: Brain health records should be established, with precision management in three categories: A (normal), B (medium-to-high risk), and C (suspected).
9	Intervention and Medical Referral	1. Non-pharmacological interventions: Regular activities including cognitive training, handicrafts, and music therapy should be conducted; individualized plans should be developed for high-risk older adults. 2. Green-channel referrals: Fast-track referral pathways should be established with specialized hospitals, along with family doctor contracting for suspected cases.
10	Family Caregiver Empowerment	1. Caregiver support: Convenient registration should be available, mutual-aid networks for family members established, and stigma alleviated. 2. Skills training: Progressive tiered caregiving training should be offered with certificates issued, along with professional guidance on home age-friendly modifications. 3. Respite care: Respite services should be provided for caregivers through professional elderly care institutions.

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